



EVIDENCE-BASED OWNER HANDBOOK · 2026 EDITION

The Senior Dog Care Field Guide

A practical system for noticing change early, protecting daily function, and working effectively with your veterinarian.

FOR FIRST-TIME SENIOR DOG CAREGIVERS

This is a care system, not a list of senior-dog tips.

The most useful thing an owner can do is recognize a meaningful change while the dog still has options.

Older dogs rarely announce illness with one dramatic sign. More often, the first signal is a small loss of function: taking two attempts to stand, turning around more slowly, drinking from a different bowl, waking at 3 a.m., or no longer following the family upstairs.

This guide teaches you how to convert those observations into a reliable baseline, a trend, and a clear decision. It does not ask you to diagnose your dog. It teaches you how to notice, measure, document, escalate, and collaborate.



The central principle

Change from your dog's normal matters more than comparison with another dog. A monthly video, a measured weight, and a precise symptom history can be more useful than a vague statement that your dog is "slowing down."

How the evidence was selected

The clinical backbone is the Merck Veterinary Manual and the American Animal Hospital Association's senior-care guidelines. Nutrition guidance is supplemented by the World Small Animal Veterinary Association and Pet Nutrition Alliance. Dental recommendations incorporate Veterinary Oral Health Council standards. Vaccination and parasite sections use AAHA, WSAVA, and Companion Animal Parasite Council guidance. Cognitive care includes the first international CCDS diagnostic and monitoring guideline, published in late 2025 and reported in 2026. Emergency and end-of-life sections use Merck, ASPCA Animal Poison Control, AAHA, and AVMA sources.

This handbook prioritizes guidelines, veterinary manuals, university veterinary sources, and professional organizations. It avoids product-sponsored claims. Where evidence is limited or individual variation is high, the guide says so.

Use this guide to improve care, not to replace veterinary care.

This guide provides general educational information. It cannot examine your dog, establish a veterinarian-client-patient relationship, diagnose disease, select a drug, or determine whether a specific treatment is safe for an individual animal.

Seek emergency veterinary care now

Go immediately for difficulty breathing, collapse or loss of consciousness, an ongoing or repeated seizure, severe or uncontrolled bleeding, major trauma, suspected poisoning, severe uncontrolled pain, heatstroke, or a rapidly swollen painful abdomen with repeated dry retching. Call the emergency hospital while preparing to leave.

Your veterinary team diagnoses

They integrate the examination, medical history, laboratory data, imaging, medications, breed risk, and response to treatment.

You supply the home evidence

You observe sleep, movement, appetite, water, bathroom habits, behavior, comfort, and function over days and weeks.

Before continuing, complete three safety actions

1

Save the emergency hospital

Store its name, phone number, and route in every caregiver's phone. Confirm whether it is open 24 hours.

2

Create one medication list

Include prescriptions, supplements, CBD or cannabis products, vitamins, topical products, dental products, and as-needed medications.

3

Record a baseline video

Film your dog rising, walking toward and away from you, turning, sitting, standing, and using normal stairs or a ramp only if safe.

CONTENTS

Read in order once. Return by symptom later.

00	The Senior Care Operating System	5
01	Aging, Risk, and the Baseline	8
02	Preventive Visits and Screening	12
03	Nutrition, Weight, and Muscle	16
04	Pain, Arthritis, and Mobility	21
05	Sleep, Breathing, and the Home	25
06	Dental and Oral Health	29
07	Recognizing Symptom Patterns	32
08	Skin, Paws, Nails, and Lumps	36
09	Vision, Hearing, and Cognition	39
10	Vaccines, Parasites, and Medications	43
11	Emergency Decisions	47
12	Quality of Life and End-of-Life Planning	51
T	Printable Owner Toolkit	54
R	References and Image Credits	67

Best first use

Set aside 30 minutes. Read the operating system, complete the 30-day setup plan, then read the chapters that match your dog's current risks. Keep the emergency card and medication list accessible.

Senior care works when observation has a cadence.

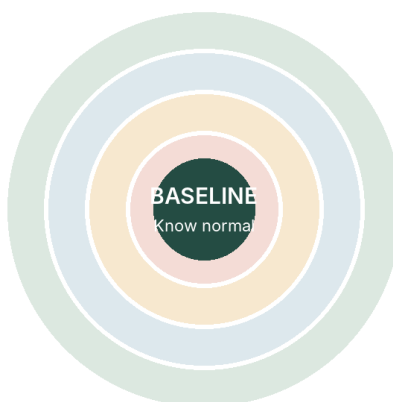
A dog can change gradually enough that the family adapts without realizing how much function has been lost. A fixed review schedule prevents “normalization of decline.”

AAHA describes senior status as approximately the final quarter of a dog's expected lifespan, which means the starting age differs by breed, body size, genetics, and health history. Merck recommends at least twice-yearly examinations for dogs in the senior stage, commonly beginning around 7 to 8 years, with individualization for earlier-aging large and giant breeds.^{1,2}

OWNER VISUAL

The Senior Care Operating System

A repeatable cadence turns vague concern into useful health evidence.

**DAILY**

Appetite, water, bathroom, movement, breathing, connection

WEEKLY

Mouth, skin, paws, nails, lumps, mobility

MONTHLY

Weight, body and muscle condition, video, dashboard

6 MONTHS

Physical exam, screening, medications, care goals

RESULT Small observations become a clear history, earlier decisions, and fewer crisis surprises.

The cadence is intentionally light. The objective is consistency, not constant surveillance.

The daily 90-second scan

Once a day, ask whether six essentials are normal: appetite, drinking, bathroom habits, movement, breathing, and connection. You are not measuring every detail. You are detecting whether today's pattern is meaningfully different from the dog's own baseline.

Function

Can the dog rise, walk, turn, toilet, eat, drink, and settle without new difficulty?

Physiology

Are breathing, thirst, urination, stool, appetite, and sleep close to normal?

Engagement

Does the dog still seek familiar people, routines, smells, food, and favorite activities?

The weekly 10-minute check

Use your hands and eyes from nose to tail. Look inside the mouth if safely tolerated, feel the body and known lumps, inspect paws and nails, note muscle over the head and hips, and watch the dog rise after rest. Weekly checks are frequent enough to catch changes while avoiding the anxiety of daily scoring.

The monthly 20-minute review

Weigh the dog under similar conditions, update body and muscle observations, repeat the baseline video, photograph known masses beside a ruler, reconcile medications, and review the good-day trend. Monthly review is where isolated observations become a pattern.

The six-month clinical review

A senior visit should not end with “everything looks fine.” It should produce a current problem list, screening decisions, nutrition and pain assessments, a monitoring plan, an explicit recheck date, and the changes that should trigger earlier contact. AAHA recommends routine senior workups once or twice yearly and provides 6- to 12-month testing intervals for common screening tests.^{1,3}

The 30-day setup result

At the end of the first month, you should have a baseline profile, a safe home route, one medication list, a body and mobility record, a veterinary screening plan, and an emergency plan. These are durable assets that make every later decision faster and more precise.

Week	Primary objective	Concrete output
1	Define normal	Profile, weight, food record, baseline video, sleep and bathroom description
2	Protect function	Traction path, stair plan, accessible water, bed assessment, nail plan
3	Reduce preventable risk	Dental routine, medication reconciliation, parasite and vaccine calendar
4	Align with the veterinarian	Senior visit brief, screening decisions, symptom thresholds, next appointment

A black dog, possibly a Labrador Retriever, is sitting in a field of tall, dry grass. The dog is looking slightly to the left of the camera with its mouth open and its pink tongue hanging out. It is wearing a silver chain collar with a small tag. The background is a soft-focus field of grass.

CHAPTER 01 · FOUNDATION

Aging, Risk, and the Baseline

Learn what “senior” means for your dog and create the reference point that makes future change visible.

Result by the end of this chapter

You will have a date-stamped baseline that describes how your dog normally eats, drinks, sleeps, moves, toilets, breathes, and engages.

Aging is a risk state, not a diagnosis.

Gray hair is an age change. Difficulty standing is a clinical change. The difference matters.

Dogs do not age on a single universal timetable. A large dog's final quarter of expected life may begin years before a small dog's. Breed-related disease risk, prior injuries, body condition, chronic medication use, and individual resilience also affect when senior-focused care should begin. The practical question is not "Is my dog officially old?" It is "Has the dog's risk changed enough that closer monitoring will improve decisions?"^{1,2}

COMMON ASSUMPTION

"Slowing down is normal."

Some change in endurance can accompany age, but new reluctance, stiffness, altered gait, loss of learned behavior, accidents, or nighttime disruption can reflect pain or disease. Treat "old age" as a prompt to investigate, not an explanation.

OWNER VISUAL

Build a Baseline Before You Need One

Record the same six views every month, ideally after a normal period of rest.

1 Rise from rest



Do not call or lure until the dog initiates movement.

2 Walk toward camera



Head, stride, symmetry, confidence.

3 Walk away



Pelvis, rear feet, drifting, toe scuffing.

4 Turn both directions



Watch the tightness and number of steps.

5 Sit and stand



Note hesitation, shifting, or repeated attempts.

6 Stairs or ramp



Only if already safe and part of normal life.

Name files by date. Compare month to month, not memory to memory.

Repeat the same sequence monthly. Consistency makes subtle asymmetry and loss of function easier to see.

The baseline has two layers

Layer 1: what the dog can do

Record function in ordinary language: rises from the living-room rug without assistance; walks 25 minutes on level ground; uses eight stairs with no pause; jumps into the sedan but not the SUV; sleeps through most nights; asks to go outside four times daily. Function statements are more useful than labels such as “active” or “stiff.”

Layer 2: how the dog does it

Note speed, symmetry, hesitation, number of attempts, breathing effort, recovery time, and emotional response. Two dogs may both climb stairs, but one climbs continuously while the other pauses, shifts weight forward, and needs encouragement.

The change equation

When deciding whether a change matters, consider four variables together: **magnitude** (how different), **duration** (how long), **functional impact** (what the dog can no longer do), and **clustering** (what else changed at the same time). A small isolated change may be monitored; several changes occurring together deserve faster review.

Write observations that a veterinarian can use

Vague	Usable
"Drinking a lot."	"For four days, the 2-liter bowl is empty by dinner instead of half full, and he has asked outside twice overnight."
"Her back legs are weak."	"After sleeping, she needs two or three attempts to stand and slips on the rear right foot when turning left."
"He is acting old."	"He no longer greets us at the door, wakes between 2 and 4 a.m., and has urinated indoors three times this week."

Create the first baseline in one session

- 1

Record identity and risk
Age or estimated age, breed or mix, body weight, sex and reproductive status, known diagnoses, prior surgeries, current medications, allergies, and family history when known.
- 2

Describe a normal day
Measure food, list treats, estimate water-bowl use, record bathroom frequency, walk duration, preferred sleep locations, and normal nighttime behavior.

3

Film function after rest

Use the six-view protocol. Keep the camera at the dog's body height, use good light, and avoid slippery surfaces that create an artificial problem.

4

Save files predictably

Use a name such as **2026-08-21_Rise-and-Walk**. Store all records in one folder that another caregiver can find.

COMPOSITE OWNER SCENARIO

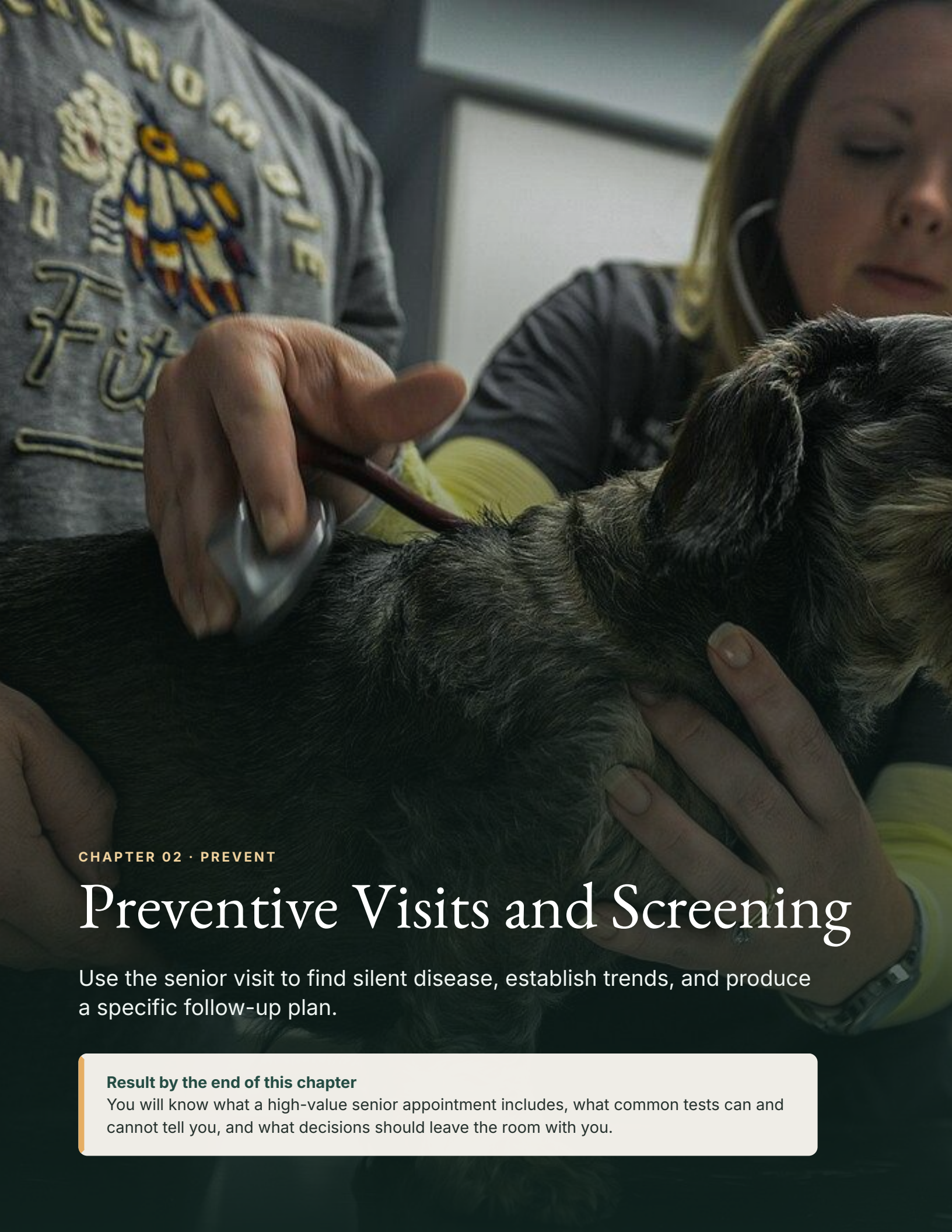
Luna's "sudden" arthritis was visible three months earlier.

Luna's family noticed she stopped jumping into the car. Reviewing monthly videos showed that she had already begun shifting weight forward and taking shorter rear-leg steps after sleep. The earlier footage gave the veterinarian a clearer progression history and helped distinguish a chronic mobility problem from an acute injury.

CHAPTER RESULT

Your baseline is complete when another caregiver could recognize a meaningful change.

Use the Baseline Profile and Monthly Dashboard in the toolkit. Update the record after a major diagnosis, surgery, medication change, or move to a new home.



CHAPTER 02 · PREVENT

Preventive Visits and Screening

Use the senior visit to find silent disease, establish trends, and produce a specific follow-up plan.

Result by the end of this chapter

You will know what a high-value senior appointment includes, what common tests can and cannot tell you, and what decisions should leave the room with you.

A senior visit is a surveillance system.

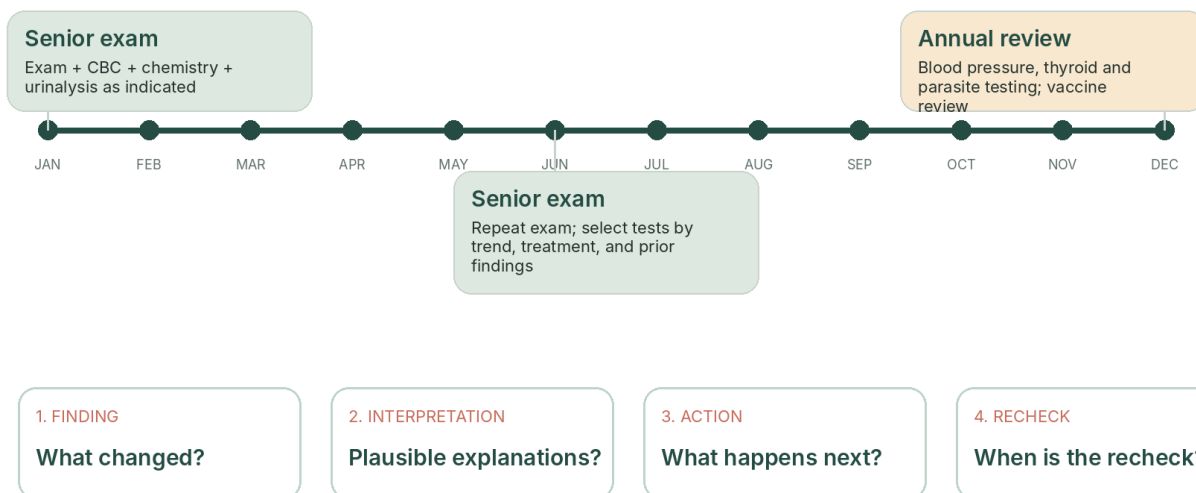
The purpose of screening is not to order every possible test. It is to detect important change before the dog reaches a crisis.

Merck advises at least twice-yearly veterinary examinations for senior dogs. AAHA recommends a routine medical workup once or twice yearly and gives a 6- to 12-month framework for common laboratory screening. Frequency changes when a dog is ill, takes medications that require monitoring, has prior abnormal results, or belongs to a higher-risk breed.^{1,2,3}

OWNER VISUAL

What a Senior Wellness Plan Should Produce

Screening is valuable when every test answers a question and leads to a follow-up plan.



A screening result is only useful when it leads to interpretation, action, and a defined recheck.

What the physical examination should cover

A senior examination should connect body systems with daily function. In addition to heart, lungs, abdomen, eyes, ears, skin, and mouth, ask for body condition, muscle condition, joint and gait assessment, pain review, blood pressure decision, medication reconciliation, and behavioral history. Bring videos because some dogs move differently at home than in a clinic.

Common screening tests, in plain language

Test	What it contributes	Typical AAHA framework for a healthy senior dog
Complete blood count	Red cells, white cells, and platelets; may reveal anemia, inflammation, infection, or blood abnormalities	Every 6 to 12 months
Blood chemistry	Organ and metabolic information, including kidney and liver markers, glucose, protein, and electrolytes	Every 6 to 12 months
Urinalysis	Kidney concentrating ability, blood, protein, glucose, sediment, and evidence that may support urinary disease	Every 6 to 12 months
Blood pressure	Identifies hypertension, which can damage organs and may accompany other disease	Generally annually
Thyroid testing	Supports evaluation of suspected thyroid disease when interpreted with signs and other tests	Generally annually
Heartworm and vector-borne testing	Detects exposure or infection according to geography and prevention strategy	Generally annually

This table summarizes AAHA guidance rather than prescribing a universal protocol. Imaging, ECG, urine protein testing, infectious-disease testing, or other diagnostics are added when history, breed, examination, or screening results justify them.³

Why blood and urine belong together

Blood values show what is circulating. Urine shows how the kidneys concentrate and what is being lost or excreted. A blood panel without a urinalysis can leave important kidney and urinary information unresolved.

Do not confuse screening with diagnosis

An abnormal screening result is a starting point. It may need confirmation, repeat testing, a urine culture, imaging, endocrine testing, blood-pressure measurement, or review of hydration and medications. A normal result also does not erase a real home symptom. The veterinarian must reconcile the laboratory data with the dog.

Ask after every abnormal value

Is this likely clinically meaningful? What are the plausible explanations? What test or recheck separates them? What change would make this urgent?

Senior anesthesia: age alone is not the decision

Older dogs may still benefit from dentistry, mass removal, imaging, or other procedures that require anesthesia. AAHA states that advanced age alone is not a contraindication. Risk assessment should instead

consider current disease, functional reserve, airway and cardiac status, laboratory findings, procedure length, pain control, monitoring, and expected benefit.³³

Useful question

Do not ask only, “Is my dog too old for anesthesia?” Ask, “What specific risks does my dog have, how will the team reduce them, and what is the likely benefit of doing or not doing the procedure?”

Prepare a one-page appointment brief

Bring the current medication and supplement list, food and quantity, weight trend, baseline comparison, symptom start date, frequency, triggers, videos, lump photos, bathroom changes, and three priority questions. This prevents the most important issue from surfacing in the last two minutes.

The appointment is complete when you leave with:

- a current problem list and what remains uncertain;
- the tests selected, what each is intended to answer, and how results will be communicated;
- medication and nutrition instructions in writing;
- the home metrics to track;
- a specific recheck date and clear earlier-contact thresholds.

CHAPTER RESULT

Your veterinary visit now has an input, a decision, and an output.

Use the Vet Visit Brief in the toolkit. After the visit, write the plan in your own words and confirm any ambiguity with the clinic.



CHAPTER 03 · NOURISH

Nutrition, Weight, and Muscle

Protect lean tissue, avoid invisible calorie creep, and choose food according to the individual dog's condition rather than the word "senior."

Result by the end of this chapter

You will have a measured feeding plan, a body and muscle baseline, and a defined recheck method for any nutrition change.

The number on the scale is only one signal.

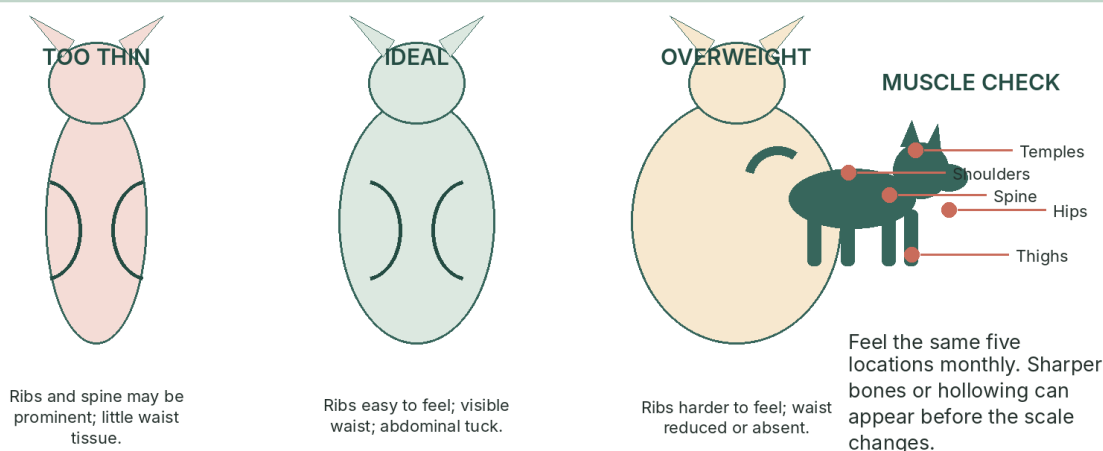
A dog can maintain the same body weight while losing muscle and accumulating fat. That is why senior nutrition assessment requires weight, body condition, and muscle condition together.

AAHA notes that there is no single AAFCO nutrient profile that defines a “senior” dog food. Healthy older dogs, overweight dogs, underweight dogs, and dogs with kidney, heart, gastrointestinal, endocrine, or cancer diagnoses may require very different diets. Choosing food by age label alone is not a clinical plan.^{1,4}

OWNER VISUAL

Weight Is Not the Same as Body Composition

A senior dog can keep the same weight while losing muscle and gaining fat.



TRACK WEIGHT + MUSCLE

Use the scale, the view from above, the side profile, and hands over muscle sites. One measure can hide what another reveals.

Body Condition Score and Muscle Condition Score answer different questions

Body Condition Score estimates body fat by looking at rib coverage, waist, abdominal tuck, and fat deposits. Muscle Condition Score evaluates muscle over the temples, shoulders, spine, hips, and thighs. A dog can be overweight and muscle-depleted at the same time, which is why simply cutting calories can worsen frailty if the plan ignores protein, disease, and activity.

Monthly body check

Weigh under similar conditions. Photograph from the side and above. Feel the five muscle sites. Write one sentence: “weight stable, ribs harder to feel, thighs less full,” or “weight down, waist unchanged, spine more prominent.” This is more informative than recording weight alone.

Do not automatically restrict protein because a dog is old

Healthy senior dogs may need adequate or even increased high-quality protein to preserve lean tissue. Protein restriction belongs to specific medical decisions, not age by itself. Merck and AAHA both emphasize individualization, and disease-specific diets should be selected with the veterinarian rather than improvised from internet advice.^{4,6}

Use a complete-diet-first food ledger

OWNER VISUAL

The Daily Food Ledger

The goal is not perfect counting. It is knowing where calories and nutrients are coming from.



MEALS

80-90%

Measured complete and balanced food

TREATS

up to 10%

Training food, chews, table food

EXTRAS

count them

Supplements, toppers, dental products

1 Measure — 2 Feed — 3 Observe — 4 Weigh — 5 Adjust with vet

A food ledger reveals where calories and nutrients come from. Treats, chews, toppers, and supplements are part of the diet.

WSAVA recommends evaluating whether the main food is complete and balanced for the dog's life stage and whether the manufacturer can provide qualified nutrition expertise, quality-control information, nutrient analysis, and research support. Marketing language such as “holistic,” “human-grade,” or “premium” does not answer those questions.⁵

As a practical rule, foods that are not complete and balanced, including treats and table foods, should usually remain a small fraction of daily intake. A 10% ceiling helps prevent calorie creep and nutrient dilution, although a veterinarian may set a different plan for an individual dog.

Measure first, then change one variable

1 **Measure the current intake**

Weigh food in grams when possible. Record every treat, chew, topper, and supplement for three representative days.

2 **Define the target**

Examples: stop weight gain, restore lost weight, preserve muscle, improve stool consistency, or support a diagnosed disease. "Eat healthier" is too vague to evaluate.

3 **Change one major factor**

Alter calories, food, treat allocation, meal format, or feeding frequency with veterinary guidance. Changing everything simultaneously makes cause and effect impossible to interpret.

4 **Recheck the result**

For a routine calorie or diet adjustment, schedule a weight and condition recheck in roughly 2 to 4 weeks unless the veterinarian sets a different interval. Record appetite, stool, vomiting, energy, and muscle.

Hydration is access plus observation

Provide fresh water in locations the dog can reach without stairs or slippery flooring. If drinking seems different, measure it for 24 hours using one known-volume container where practical. Account for multiple pets, wet food, outdoor water, heat, and activity. A single number does not diagnose disease, but a persistent change from baseline is valuable clinical evidence.

Diet change should not delay a diagnosis

Unplanned weight loss, visible muscle loss, persistent appetite reduction, increased thirst or urination, repeated vomiting, diarrhea, difficulty chewing, or marked food refusal deserves veterinary assessment. Do not spend weeks cycling through foods while the underlying problem progresses.

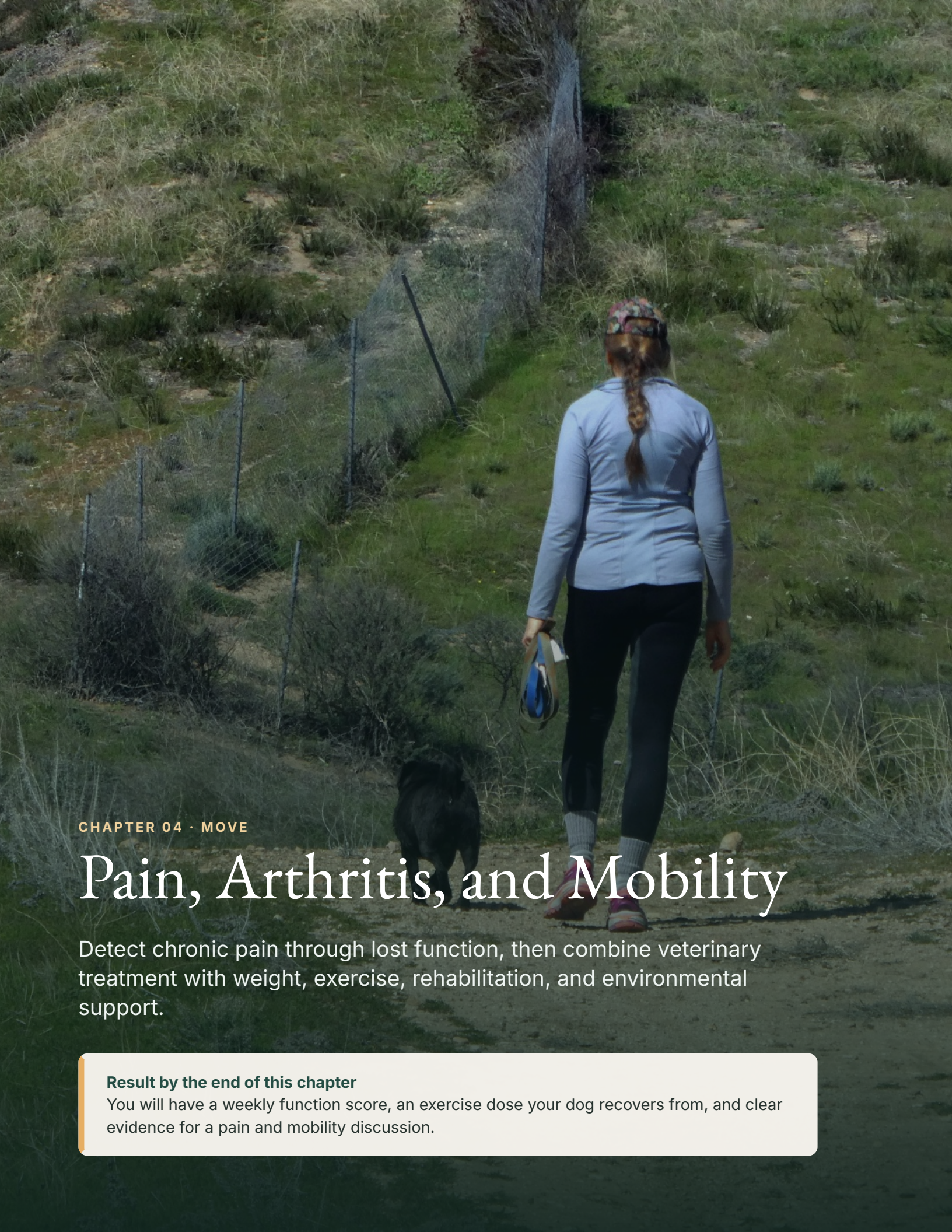
Supplements require the same scrutiny as medications

"Natural" does not mean safe or effective. Supplements can duplicate ingredients, interact with drugs, alter bleeding risk, contribute calories, or create gastrointestinal effects. Bring the actual container or a clear label photo to every senior review.

CHAPTER RESULT

Your feeding plan is measurable.

You know exactly what the dog receives, what outcome the plan is designed to produce, and when weight, body condition, muscle, appetite, and stool will be rechecked.

A person with a braid and a colorful headband, wearing a light blue long-sleeved shirt and black leggings, is walking away from the camera on a dirt trail. They are holding a blue and yellow bag in their right hand. A black dog is walking ahead of them. The trail is surrounded by dry grass and shrubs, with a wire fence visible on the left side.

CHAPTER 04 · MOVE

Pain, Arthritis, and Mobility

Detect chronic pain through lost function, then combine veterinary treatment with weight, exercise, rehabilitation, and environmental support.

Result by the end of this chapter

You will have a weekly function score, an exercise dose your dog recovers from, and clear evidence for a pain and mobility discussion.

Chronic pain often looks like a smaller life.

A dog may stop following the family, greeting at the door, climbing into the car, or exploring outside long before anyone hears a cry.

Osteoarthritis is a progressive whole-joint disease and a common source of chronic pain in dogs. Lameness can be subtle even when disease is substantial. AAHA pain guidelines emphasize owner observation because the family sees repeated daily function that a clinic cannot reproduce in a short examination.^{7,8}

Ask what stopped, not only what hurts

“No longer jumps into the car” and “turns home after 12 minutes” are functional losses. They may reflect pain even when the dog still eats, wags, and does not limp continuously.

OWNER VISUAL

Measure Function, Not Just Limping

Score the same five tasks weekly. A trend is more useful than a single “good day.”

RISE	Gets up after rest	0 normal	1 slight change	2 clear difficulty	3 unable / distress
WALK	Keeps normal pace	0 normal	1 slight change	2 clear difficulty	3 unable / distress
TURN	Turns without extra steps	0 normal	1 slight change	2 clear difficulty	3 unable / distress
STAIRS	Uses safely and willingly	0 normal	1 slight change	2 clear difficulty	3 unable / distress
SETTLE	Lies down and sleeps comfortably	0 normal	1 slight change	2 clear difficulty	3 unable / distress

24-HOUR RULE

More stiffness tomorrow means reduce the dose and discuss the pattern.

Score the same tasks under similar conditions. Share the trend and representative videos with the veterinary team.

Five high-value functions

Track rising after rest, normal walking pace, turning, stairs or vehicle access, and the ability to lie down and settle. Score each from 0 to 3 weekly. The score is not a diagnosis and should not be used to adjust prescription medication independently. Its purpose is to show whether function is stable, improving, or deteriorating.

Exercise is a dose

Merck supports regular mild-to-moderate activity for dogs with osteoarthritis while cautioning against excessive strenuous exercise. Consistency usually matters more than intensity. A long weekend hike cannot compensate for six inactive days, and an enthusiastic dog can exceed what painful joints tolerate before showing the consequences later.⁹

1

Start below the failure point

Choose a duration and terrain the dog completes without lagging, repeated stopping, heavy recovery, or loss of form.

2

Prefer regular low-impact movement

Shorter, more frequent walks, sniffing, controlled turns, gentle strengthening, and rehabilitation exercises may be better tolerated than repetitive high-impact fetch or sudden running.

3

Apply the 24-hour response rule

Observe the dog later the same day and the next morning. More stiffness, slower rising, new reluctance, or prolonged fatigue means the dose may have exceeded current capacity.

4

Change gradually

Increase one variable at a time: duration, slope, speed, or complexity. A rehabilitation professional can design exercises when weakness, neurologic disease, or postoperative recovery complicates the plan.

Mobility treatment is usually multimodal

A high-quality plan may combine weight optimization, prescription analgesia or anti-inflammatory medication, rehabilitation, home traction, nail care, supportive devices, controlled activity, and selected adjuncts. No single bed, supplement, injection, or exercise replaces a complete assessment.

Reduce avoidable load

Maintain an appropriate body condition, shorten overgrown nails, trim excess paw hair, and prevent repeated leaps from high furniture or vehicles.

Increase usable confidence

Create a non-slip path, use stable ramps, improve lighting, protect stairs, and place resources on the floor where the dog spends most time.

Pain can alter behavior

Restlessness, inability to settle, nighttime waking, withdrawal, irritability, aggression when touched, accidents, and reduced social interaction can all accompany pain. A sudden “personality problem” in a senior dog warrants a medical and pain review before being treated only as a training issue.

Never give a human pain medicine unless a veterinarian specifically directs it

Human NSAIDs and other medications can cause severe gastrointestinal, kidney, liver, neurologic, or bleeding complications in dogs. Senior dogs may have silent organ disease that further changes risk.

COMPOSITE OWNER SCENARIO

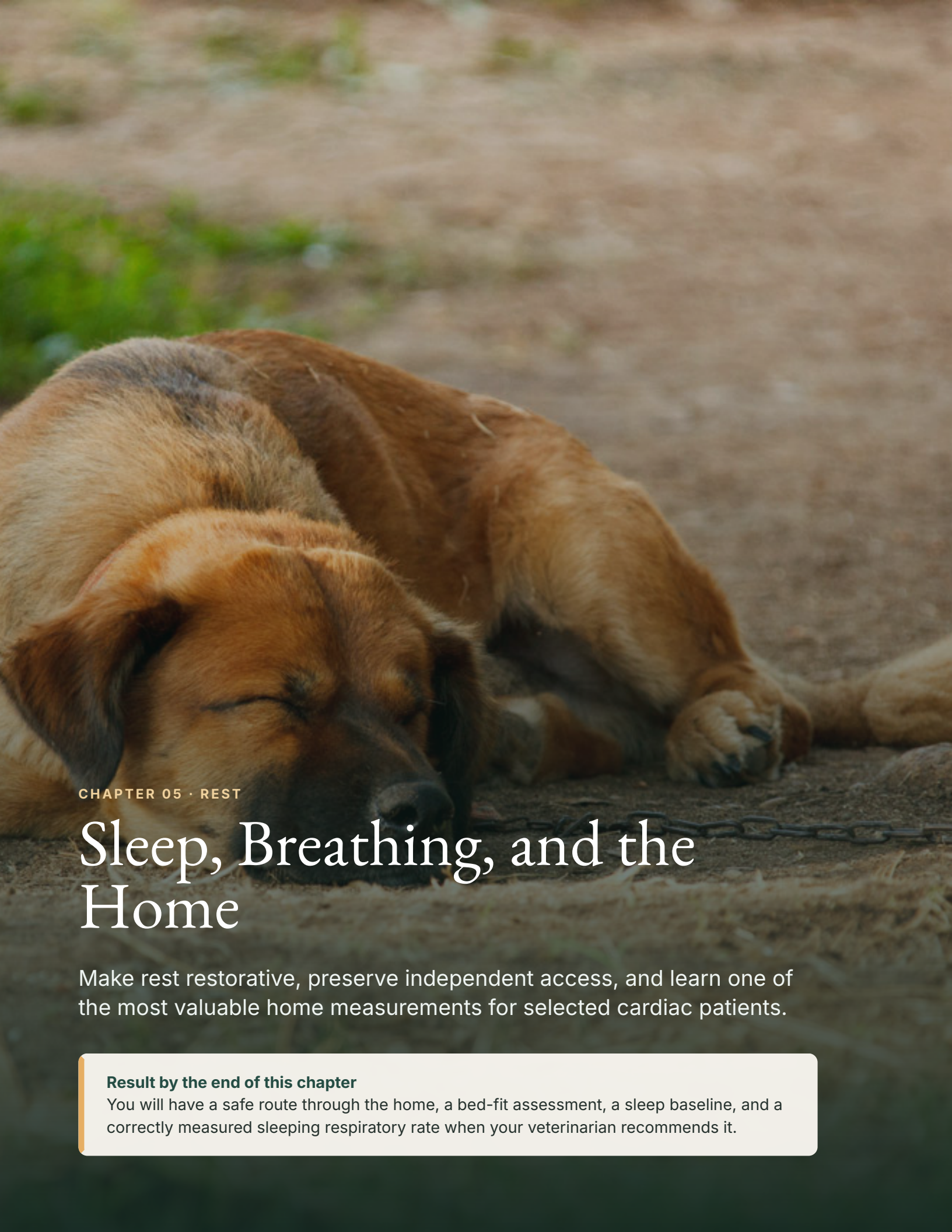
Cooper still walked, but the recovery told the story.

Cooper completed a 35-minute walk without limping, so the family assumed the distance was appropriate. The next morning he took four attempts to stand and avoided breakfast until the bowl was moved closer. Reducing the walk to two shorter sessions and beginning a veterinary pain and rehabilitation plan improved his weekly function score.

CHAPTER RESULT

You can show whether function is changing.

Use the Weekly Mobility Score and monthly video. Contact the veterinarian when the trend worsens, a new task becomes impossible, pain disrupts sleep or toileting, or weakness develops suddenly.



CHAPTER 05 • REST

Sleep, Breathing, and the Home

Make rest restorative, preserve independent access, and learn one of the most valuable home measurements for selected cardiac patients.

Result by the end of this chapter

You will have a safe route through the home, a bed-fit assessment, a sleep baseline, and a correctly measured sleeping respiratory rate when your veterinarian recommends it.

The home can either preserve function or quietly take it away.

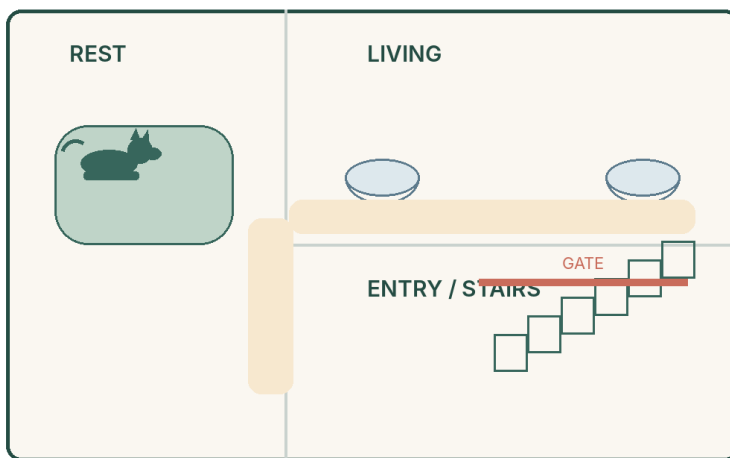
A slippery hallway can make water inaccessible. A tall bolster can make a comfortable bed impossible to enter. A dark staircase can turn mild vision loss into a fall risk.

AAHA recommends environmental modification as a core component of senior care, including secure rugs, ramps, supportive bedding, nightlights, harnesses, and protection around stairs or pools. The aim is not to make the house look medical. It is to preserve independent access to essentials.¹

OWNER VISUAL

Design a Safe Route Through the Home

The best modification is the one that preserves independent access to essentials.



- 1 Stable, supportive bed**
Dog can stretch, turn, enter, and rise without slipping.
- 2 Traction path**
Rugs connect bed, water, door, and family.
- 3 Multiple water points**
No stairs required to drink.
- 4 Gate and nightlight**
Reduce falls when vision or cognition changes.

Map the route between bed, water, food, toilet access, and family. Fix the highest-risk gap first.

Assess the bed by watching the dog use it

A senior bed should be stable, adequately padded, large enough for the dog's preferred positions, easy to enter, and easy to clean. No material is universally "orthopedic." The useful question is whether the dog can step in, turn, lower the body, remain comfortable, and rise without the bed sliding or the body contacting a hard floor.

Signs the setup works

The dog chooses the bed, changes position without repeated effort, sleeps for meaningful periods, and rises without slipping.

Signs to reassess

Repeated circling, body hanging over the edge, difficulty crossing a bolster, constant relocation, panting at rest, or struggling to rise.

Sleep change is data

Many senior dogs sleep more, but the direction and context of change matter. New nighttime pacing may reflect pain, urinary urgency, cognitive dysfunction, anxiety, sensory loss, breathing disease, medication effects, or environmental discomfort. Record when the waking begins, what the dog does, whether toileting helps, and whether daytime sleep has increased.

Sleeping respiratory rate

For dogs with known or suspected heart disease, veterinarians often use sleeping respiratory rate as a home trend. Merck notes that normal dogs generally have a sleeping respiratory rate below 30 breaths per minute, but the most useful threshold is the one set for the individual dog. A persistent increase from baseline is more important than one isolated count.¹¹

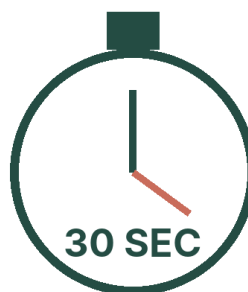
OWNER VISUAL

Count Sleeping Breaths Correctly

Use this only when your dog is truly asleep, comfortable, and not panting or actively dreaming.



ONE RISE + ONE FALL = ONE BREATH



then multiply by 2

1 Wait for sleep

2 Count chest cycles

3 Record date + result

4 Watch the trend

Count only during true sleep. One complete rise and fall of the chest equals one breath.

1

Wait for true sleep

The dog should be comfortable, not panting, not recently active, and not visibly twitching or breathing irregularly during a dream.

2

Count one chest cycle as one breath

Count for 30 seconds and multiply by two, or count a full minute. Video can help when movement is subtle.

3

Record context

Date, time, result, medication timing, room temperature, cough, and any breathing effort. Use the same method each time.

4

Follow the veterinarian's threshold

Do not change heart medication from the number alone. Contact the managing clinic for a sustained rise or any new cough, effort, restlessness, weakness, or reduced stamina.

Do not count while the dog is struggling to breathe

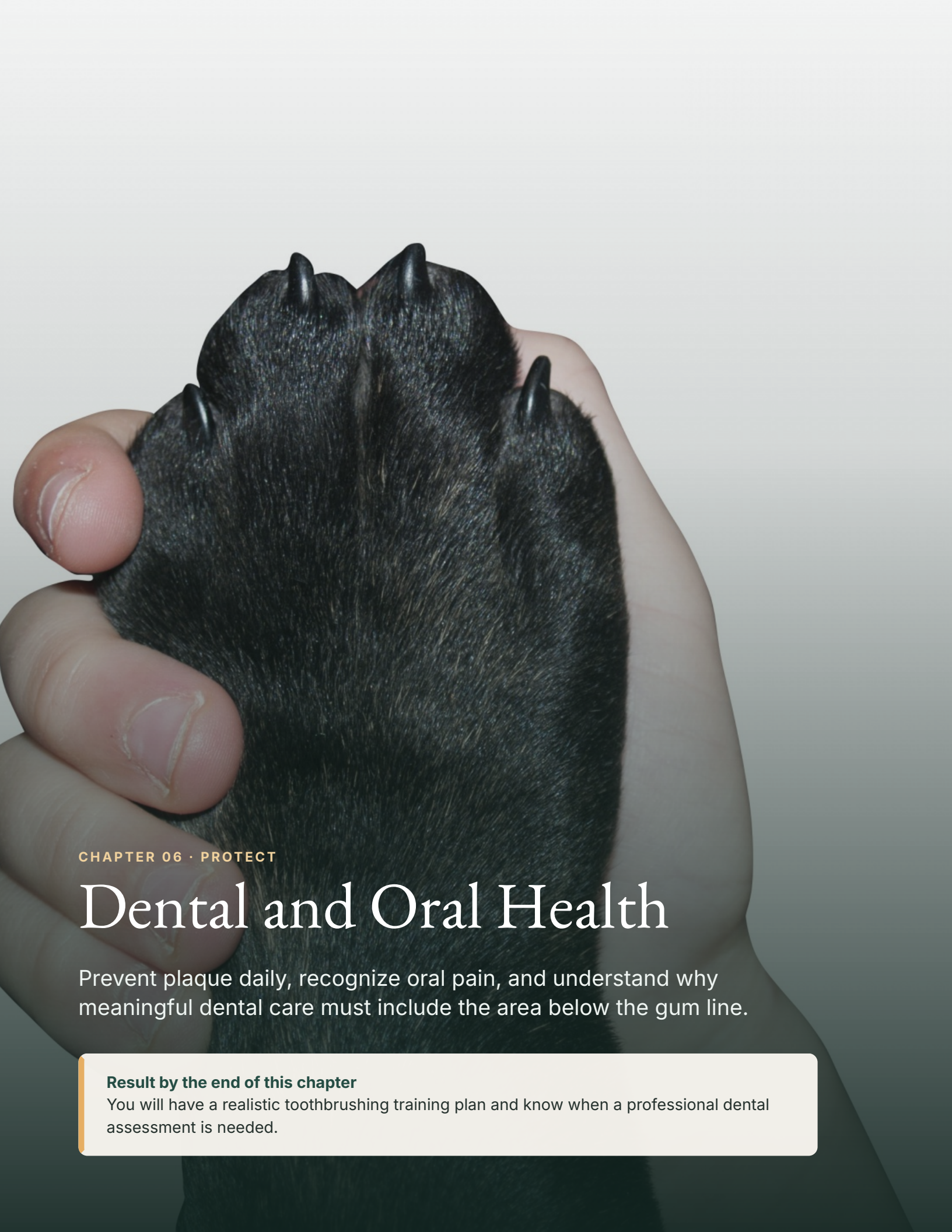
Open-mouth breathing at rest, obvious abdominal effort, blue or gray gums, inability to settle, collapse, or severe distress is an emergency. Minimize stress and leave for emergency care.

Temperature and airways

Older dogs with heart, lung, airway, endocrine, obesity, or neurologic problems may tolerate heat poorly. Large senior dogs with new noisy breathing, voice change, dry cough, exercise intolerance, or heat distress need prompt airway assessment because laryngeal paralysis and the broader geriatric-onset laryngeal paralysis polyneuropathy complex are important possibilities.¹⁹

CHAPTER RESULT**The home now supports the dog's remaining ability.**

Complete a room-by-room route check, observe actual bed use, and record sleep changes. Use the Respiratory Rate Log only when it is relevant to the dog's veterinary plan.



CHAPTER 06 • PROTECT

Dental and Oral Health

Prevent plaque daily, recognize oral pain, and understand why meaningful dental care must include the area below the gum line.

Result by the end of this chapter

You will have a realistic toothbrushing training plan and know when a professional dental assessment is needed.

A dog can keep eating with a painful mouth.

Appetite alone cannot prove that teeth and gums are comfortable.

Periodontal disease begins with plaque and inflammation around the teeth, then can damage the supporting tissues and lead to pain, infection, and tooth loss. Much of the clinically important disease is below the gum line and cannot be judged from a quick look at visible tartar.¹²

Watch the mechanics of eating

Bad breath, red or bleeding gums, dropping food, chewing on one side, slower meals, reluctance with hard food, pawing at the mouth, facial swelling, drooling, or a new preference for soft food deserves assessment. A dog may still approach the bowl enthusiastically because hunger and pain can exist together.

OWNER VISUAL

Build Toothbrushing in Four Small Steps

The objective is calm daily access to the outer tooth surfaces, not a wrestling match.



DO NOT SUBSTITUTE "ANESTHESIA-FREE CLEANING" FOR A VETERINARY DENTAL ASSESSMENT.

Disease below the gum line cannot be evaluated or treated by surface scaling alone.

Training in small increments improves cooperation. Stop before distress and resume at an easier step.

Brushing works because plaque reforms quickly

Merck identifies daily brushing as the most effective home method for plaque control, with every-other-day brushing still helpful when daily care is not feasible. Use a dog-safe toothpaste and a soft brush. Focus on the outer surfaces where plaque commonly accumulates. Human toothpaste is not appropriate.¹²

Home care cannot remove established disease below the gum line

Professional dental care may require anesthesia for a complete oral examination, probing, dental radiographs, scaling above and below the gum line, polishing, and treatment of diseased teeth. AAHA and other veterinary dental organizations do not recommend anesthesia-free surface cleaning as a substitute because it cannot adequately evaluate or treat subgingival disease and can create a false sense of security.^{13,35}

Anesthesia decision for a senior dog

The risk discussion should include the dog's current heart, airway, kidney, liver, endocrine, and neurologic status; pre-anesthetic testing; monitoring; procedure duration; pain control; and the consequences of leaving oral disease untreated. Age alone is not the deciding factor.³³

Choose adjunct products by evidence, not packaging

Dental diets, chews, water additives, and other products vary widely. The Veterinary Oral Health Council awards its seal to products that meet defined plaque or tartar-control criteria. A VOHC-accepted product can supplement brushing, but it does not replace a professional assessment when disease is present.¹⁴

Avoid tooth-fracture risks

Extremely hard objects, including some bones, antlers, hooves, and rigid chews, can fracture teeth. A product marketed as "natural" or "long-lasting" is not automatically safe for an older mouth.

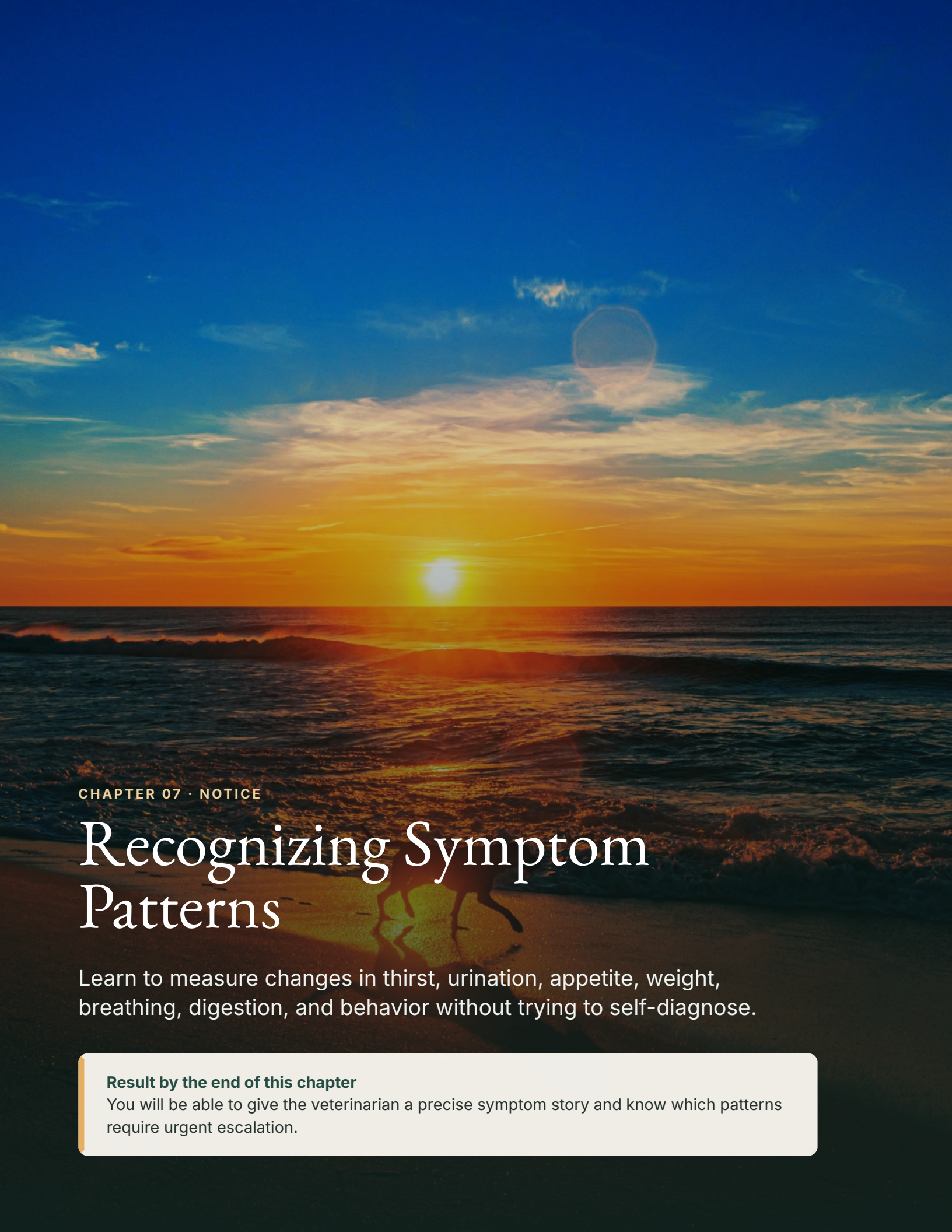
Build the routine around consent and predictability

Practice at the same time, use the same location, keep sessions short, and reward calm participation. If the dog suddenly resists handling that was previously tolerated, consider pain rather than stubbornness. Do not force the mouth open in a dog that may bite or has severe oral pain.

CHAPTER RESULT

Home dental care has a behavior plan and a clinical boundary.

You know what brushing can prevent, what it cannot treat, which signs suggest pain, and why a complete veterinary dental evaluation may still be necessary.

A full-page background image showing a sunset over the ocean. The sun is low on the horizon, creating a bright orange and yellow glow that reflects on the water. The sky is a deep blue with some wispy clouds. In the foreground, a dark silhouette of a dog is running on a sandy beach, its reflection visible in the shallow water.

CHAPTER 07 · NOTICE

Recognizing Symptom Patterns

Learn to measure changes in thirst, urination, appetite, weight, breathing, digestion, and behavior without trying to self-diagnose.

Result by the end of this chapter

You will be able to give the veterinarian a precise symptom story and know which patterns require urgent escalation.

A symptom becomes useful when it has context.

“He is drinking more” is a concern. “He emptied two measured liters, woke twice to urinate, and lost weight despite eating normally” is a clinical pattern.

Many diseases share outward signs. Kidney disease, diabetes mellitus, hyperadrenocorticism, urinary disease, medication effects, and other conditions can all change thirst or urination. Coughing can arise from airway, lung, heart, infectious, or other problems. The owner's job is to describe the pattern accurately so the veterinarian can choose the next test.^{15,16,17,18}

OWNER VISUAL

Describe a Pattern, Not a Diagnosis

The same symptom can have many causes. A clean pattern helps the veterinarian narrow the next test.

PATTERN	WHAT TO MEASURE	WHY IT MATTERS
Drinking + urinating more	Bowl refills, overnight trips, accidents, urine volume	Kidney, endocrine, urinary, medication and other causes differ
Eating more + losing weight	Measured food, weekly weight, stool quality	Can occur with diabetes or poor nutrient absorption, among other causes
Cough + lower stamina	When cough occurs, walk duration, sleeping breath trend	Helps separate airway, lung, cardiac and other patterns
Night pacing + accidents	Time of night, pain signs, urine changes, disorientation	Pain, urinary disease, sensory loss and cognition can look similar
New noisy breathing	Sound, trigger, heat response, voice change	Older large dogs may need urgent airway assessment

Use: what changed + when + frequency + trigger + what else changed + video/photo if visible.

Do not attach a disease label. Measure the pattern and its companions.

Increased thirst and urination

Chronic kidney disease may first appear as increased urine volume and thirst before advanced signs such as appetite loss, vomiting, weight loss, lethargy, and dehydration. Diabetes can produce increased thirst and urination, weight loss, and sometimes increased appetite. Hyperadrenocorticism can produce thirst, urination, appetite change, panting, weakness, a pot-bellied appearance, and skin change. These overlaps are exactly why testing is necessary.^{15,16,18}

1

Measure the water source

Use a known-volume container for 24 hours where practical. Record every refill, remaining water, wet food, other pets, outdoor access, exercise, and temperature.

2

Describe urine function

Frequency, estimated volume, urgency, overnight trips, straining, blood, odor, licking, and indoor accidents. Distinguish frequent small attempts from genuinely larger volumes.

3

Record associated change

Appetite, weight, vomiting, stool, energy, panting, medication timing, and behavior.

Straining is not the same as increased urination

Repeated attempts with little output, obvious pain, blood, or inability to pass urine needs prompt veterinary contact. Do not wait for a 24-hour measurement.

Appetite and weight are a pair

Measure what is offered and what remains. Increased appetite with weight loss suggests a different pattern from reduced appetite with weight loss. Difficulty chewing, nausea, organ disease, endocrine disease, gastrointestinal disease, pain, medication effects, and cancer can all influence intake or body weight.

MEASURE**grams**

Food offered, eaten, and left. Cups vary; a kitchen scale is more consistent.

COMPARE**weekly**

Use the same scale and similar timing when a trend is under review.

ESCALATE**early**

Unplanned loss, muscle loss, or persistent appetite change deserves assessment.

Vomiting is not the same as regurgitation

Vomiting usually includes nausea and abdominal contractions. Regurgitation is often passive and may bring up undigested food or water without retching. Video is extremely useful because the distinction changes the diagnostic approach. Repeated episodes, blood, abdominal pain, lethargy, inability to keep water down, or a rapidly deteriorating dog needs prompt care.

Cough, breathing, and stamina

Record whether coughing occurs at night, after drinking, with excitement, during exercise, or at rest. Note voice change, noisy inhalation, gagging, exercise tolerance, sleeping respiratory rate, and recovery. Any obvious effort or distress is an emergency, not a home-monitoring project.

New house-soiling is a medical clue

Accidents can reflect increased urine production, urinary disease, incontinence, pain, mobility loss, neurologic disease, sensory change, anxiety, or cognitive dysfunction. Record whether the dog signals, reaches the door, postures normally, seems aware afterward, and can physically access the toilet area.

COMPOSITE OWNER SCENARIO

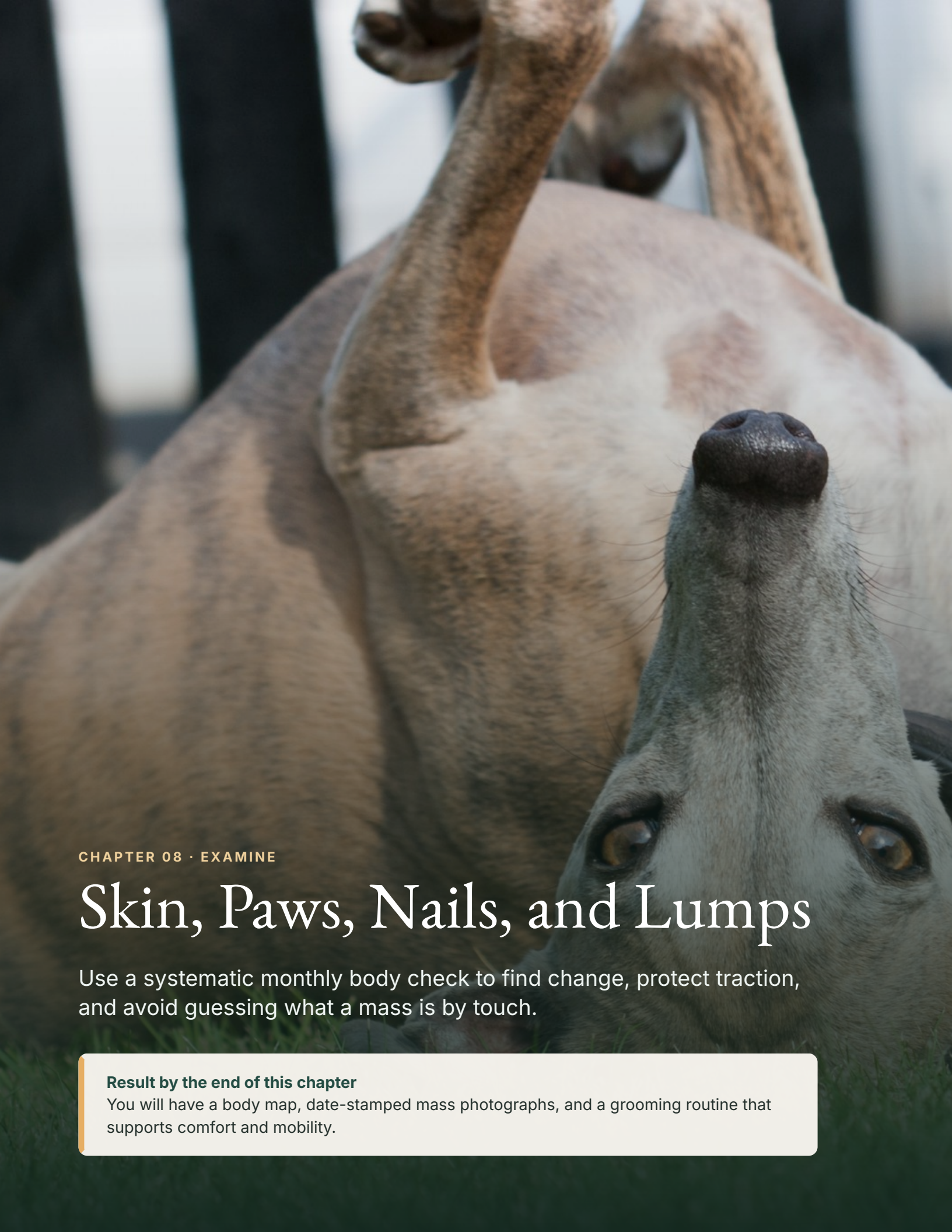
"Dementia" was actually three overlapping problems.

A senior dog began pacing at night and urinating indoors. The log showed large water intake, difficulty rising, and accidents concentrated near the back door. The veterinary workup identified a medical cause of increased urination plus arthritis that slowed access outside. Treating both changed the nighttime pattern, preventing an inaccurate cognition-only conclusion.

CHAPTER RESULT

Your symptom history is specific enough to change the diagnostic plan.

Record start date, frequency, trigger, duration, functional effect, associated signs, and evidence. Call earlier when multiple systems change together or the dog is deteriorating.



CHAPTER 08 • EXAMINE

Skin, Paws, Nails, and Lumps

Use a systematic monthly body check to find change, protect traction, and avoid guessing what a mass is by touch.

Result by the end of this chapter

You will have a body map, date-stamped mass photographs, and a grooming routine that supports comfort and mobility.

Your hands can find a change that fur hides.

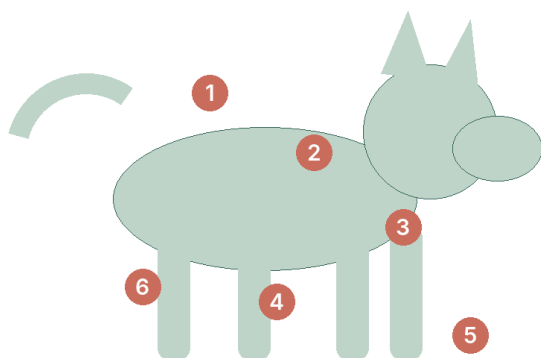
A monthly nose-to-tail check is not a cancer test. It is a change-detection method.

Skin tumors can look like small lumps, hairless patches, discolored areas, rashes, or nonhealing ulcers. Because appearance varies widely, Merck recommends veterinary identification using microscopic examination of cells or tissue, commonly fine-needle aspiration or biopsy.²⁰

OWNER VISUAL

The Monthly Nose-to-Tail Check

Photograph and map every new mass. Appearance and feel alone cannot determine what it is.



RECORD

Location	Use a body map
Size	Measure in millimeters
Surface	Smooth, ulcerated, hairless
Mobility	Moves under skin or fixed
Change	Compare date-stamped photos
Next step	Ask whether sampling is indicated

NEW OR CHANGING MASS: VET REVIEW

Map, measure, photograph, and ask whether sampling is indicated. Do not rely on feel alone.

Use the same examination route every month

Begin at the nose and lips, then check the jawline, ears, neck, shoulders, chest, armpits, front legs, paws, back, abdomen, groin, rear legs, hips, tail, and underside. Consistency prevents gaps. Stop if the dog shows pain or fear, and do not manipulate a fragile, bleeding, or ulcerated mass.

What to record about a mass

Location, length and width in millimeters, height if practical, color, surface, hair loss, ulceration, discharge, tenderness, mobility under the skin, and date first observed. Photograph beside a ruler with the dog in the same position. A "soft movable lump" may be benign, but only sampling can determine the cell type.

Why sampling before removal can matter

Fine-needle aspiration can identify some benign masses and help a surgeon plan appropriate margins when a tumor is malignant. Assuming every soft mass is a lipoma can delay the correct first surgery.²⁰

Cancer awareness without panic

Merck lists growing or changing lumps, enlarged lymph nodes, unexplained bleeding, persistent sores, unusual odor, dry cough, recurrent vomiting or diarrhea, weight loss, appetite reduction, loss of activity, pain, lameness, stiffness, and difficulty breathing, chewing, or swallowing among changes that deserve investigation. None is diagnostic of cancer by itself.²¹

Paws and nails directly affect mobility

Long nails can change foot placement and reduce traction. Excess hair between pads may worsen slipping. Check for pad cracks, foreign material, swelling between toes, broken nails, redness, licking, and asymmetric nail wear. Scuffed nails can be a clue that the dog is dragging a foot.

Grooming is a comfort intervention

Older dogs may be less able to groom, reposition, or remain dry. Mats can pull skin and hide sores. Urine or stool contamination can cause irritation. Adjust grooming position to protect painful joints, use shorter sessions, and consider mobile or veterinary-supported grooming for dogs that cannot stand comfortably.

Call promptly

A rapidly enlarging, painful, bleeding, ulcerated, discharging, or function-limiting mass; facial swelling; oral bleeding; or a mass affecting breathing, eating, vision, or toileting.

Do not “watch forever”

Set a review date for any new mass. If the plan is observation, ask what measurable change will trigger sampling.

CHAPTER RESULT

Every known mass has a location, measurement, image, and next decision.

Use the Body and Lump Map in the toolkit. Bring the map to the veterinarian instead of trying to remember which bump changed.

A black dog, possibly a Labrador Retriever, is sitting in a field of tall, dry grass. The dog is looking slightly to the left of the camera with its mouth open and its pink tongue hanging out. It is wearing a silver chain collar. The background is a soft-focus field of grass.

CHAPTER 09 · UNDERSTAND

Vision, Hearing, and Cognition

Separate sensory and medical change from cognitive decline, then build a predictable environment that preserves confidence.

Result by the end of this chapter

You will have a behavior baseline, a DISHAA log, and a plan to screen cognitive change rather than dismiss it as old age.

Behavior change is a symptom until proven otherwise.

Pacing at night can be cognition, but it can also be pain, urinary urgency, sensory loss, endocrine disease, medication effects, or neurologic disease.

Canine cognitive dysfunction syndrome is a chronic, progressive, age-associated neurodegenerative syndrome that affects daily life through changes in disorientation, social interaction, sleep, house-soiling and learning, activity, anxiety, and fear. The first international diagnosis and monitoring guideline was published in late 2025.²²

OWNER VISUAL

Cognitive Change Often Appears as a Routine Problem

Track the pattern, then let the veterinary workup separate cognition from medical mimics.



DAY

naps, exploration, social contact, normal routines



NIGHT

pacing, waking, vocalizing, accidents, difficulty settling



Disorientation



Interaction change



Sleep-wake change



House-soiling / learned behavior



Activity change



Anxiety

Starting around age 10, ask your veterinarian about structured cognitive screening every 6 months.

DISHAA organizes caregiver observations. It does not replace the physical, neurologic, orthopedic, and laboratory evaluation needed to exclude alternatives.

Screen before the change becomes disruptive

The 2026 guideline communication recommends routine senior behavior surveys beginning around age 7. When abnormalities are reported, a more detailed CCDS scale and follow-up every six months are recommended. Beginning at age 10, the working group recommends a CCDS scale every six months for all dogs.²²

Ask your veterinary clinic which validated caregiver questionnaire it uses. The value is not a one-time label. Repeated screening shows trajectory and may identify mild change that owners otherwise attribute to age.

Medical mimics must be addressed

A cognition assessment should include history, physical examination, orthopedic and neurologic evaluation, and laboratory work, with advanced imaging considered when appropriate. The new guidelines describe a practical level-one diagnosis based on progressive DISHAA signs plus exclusion or management of relevant alternative causes, and a higher-certainty level involving MRI and cerebrospinal-fluid assessment.²²

Why this matters

A dog with painful hips and reduced hearing may appear withdrawn, startle easily, and avoid stairs. A dog with increased urine production may wake and have accidents. Treating these problems can improve behavior even when cognitive change is also present.

Support vision and hearing without creating dependence

Hearing change

Use visible hand cues, approach within sight, create gentle floor vibration, and wake the dog without sudden touch near the face. Do not interpret failure to respond as defiance.

Vision change

Keep furniture stable, maintain clear pathways, use nightlights and contrast, gate unsafe stairs, and announce your presence. Sudden vision loss is urgent.

Create a cognition-supportive routine

Predictable meal, walk, toilet, medication, and sleep times reduce the amount of problem-solving required. Maintain gentle social contact and simple enrichment: sniff walks, easy food searches, familiar cues, and short choice-based games. The goal is engagement without frustration or exhaustion.

1

Track the time pattern

Day versus night, relation to meals and toileting, duration, triggers, and whether redirection works.

2

Film representative behavior

Staring, getting stuck, pacing, unusual route-finding, or altered interaction may not occur in the clinic.

3

Protect sleep

Use a consistent evening routine, accessible toilet route, comfortable temperature, pain plan, and low-level nightlight. Do not begin sedating products without veterinary review.

4

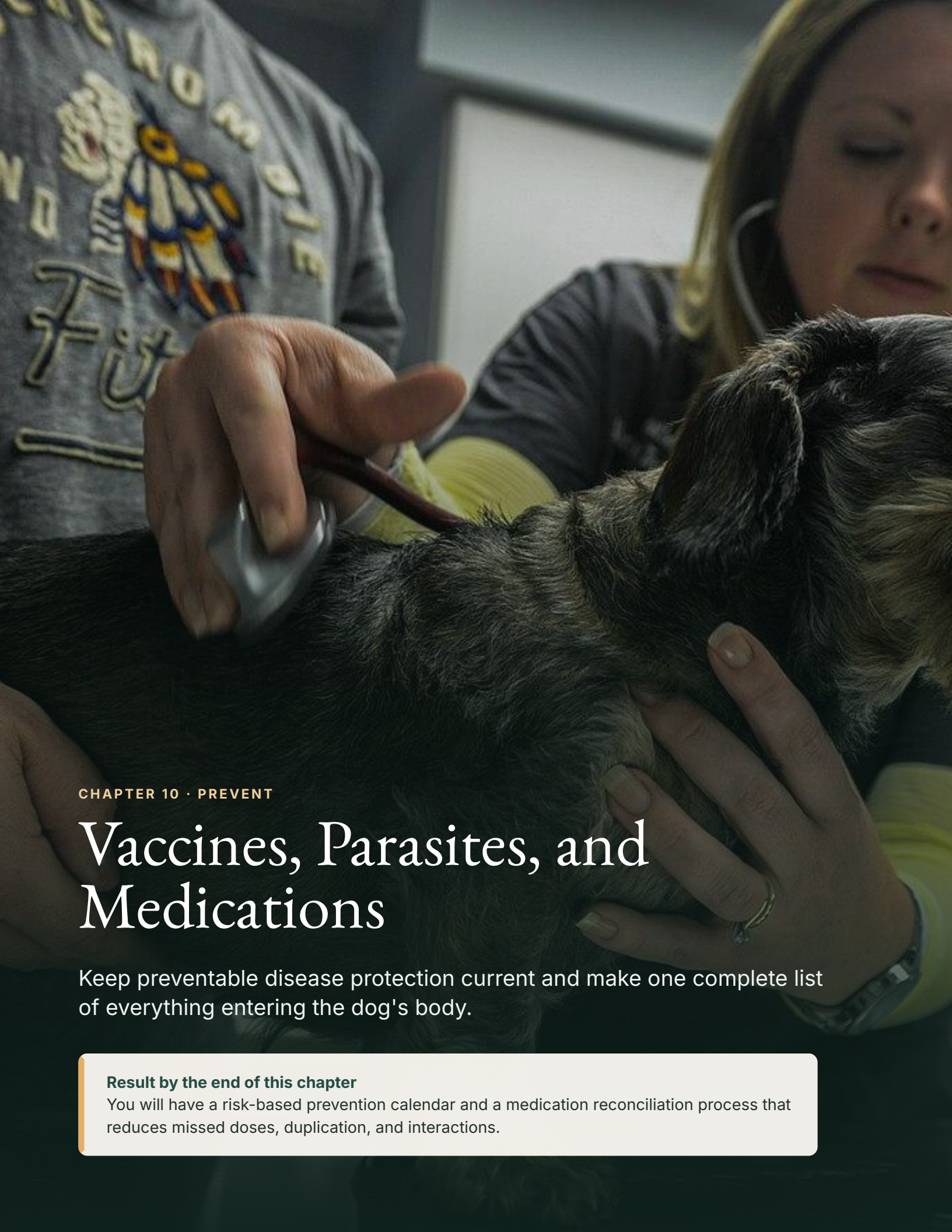
Review progression every six months or sooner

Update the DISHAA log and quality-of-life impact. Earlier review is warranted for sudden change, neurologic deficits, seizures, collapse, severe disorientation, or rapid decline.

CHAPTER RESULT

Behavior is now monitored as health data.

You can distinguish the pattern, identify functional impact, reduce environmental stress, and ask for structured screening rather than accepting "just old age."



CHAPTER 10 · PREVENT

Vaccines, Parasites, and Medications

Keep preventable disease protection current and make one complete list of everything entering the dog's body.

Result by the end of this chapter

You will have a risk-based prevention calendar and a medication reconciliation process that reduces missed doses, duplication, and interactions.

Older does not mean unexposed.

A senior dog can still acquire rabies, leptospirosis, heartworm, tick-borne disease, fleas, and intestinal parasites. Prevention should change with risk, not disappear with age.

AAHA and WSAVA recommend vaccination plans based on core protection, legal requirements, prior history, health, geography, and exposure. Current AAHA guidance includes distemper, adenovirus, parvovirus, rabies, and leptospirosis as core components for dogs, with other vaccines selected by risk.^{24,25}

OWNER VISUAL

The Protection Calendar

Age changes risk and tolerance, but it does not make infection, parasites, or medication errors disappear.

EVERY DAY

- 1 Give medication as prescribed
- 2 Keep fresh water accessible
- 3 Observe appetite, stool, and urine

Record completion date

EVERY MONTH

- 1 Confirm medication inventory
- 2 Check labels and expiration dates
- 3 Review parasite-prevention dosing

Record completion date

EVERY 6 MONTHS

- 1 Senior exam + medication review
- 2 Bring every supplement and OTC product
- 3 Review vaccine and parasite risk

Record completion date

EVERY YEAR

- 1 Heartworm test
- 2 Risk-based parasite testing
- 3 Vaccines and legal requirements

Record completion date

ONE LIST Prescription drugs + supplements + CBD/cannabis + topical products + dental products.

A single calendar reduces gaps when several caregivers, prescriptions, and prevention products are involved.

Vaccination is an individual risk-benefit decision

Discuss immune status, previous reactions, travel, boarding, grooming, daycare, wildlife, standing water, rodents, local outbreaks, and legal rabies requirements. Chronic disease may change timing or product selection, but senior status alone is not a reason to stop protection without veterinary review.

Parasite prevention is usually year-round

The Companion Animal Parasite Council recommends year-round broad-spectrum parasite control tailored to lifestyle and geography, with annual heartworm testing and appropriate fecal or vector-borne disease testing. Local risk can change with travel, climate, wildlife, and movement of infected animals.^{26,27}

Medication reconciliation prevents invisible errors

Senior dogs often accumulate prescriptions from primary care, specialists, emergency hospitals, rehabilitation providers, and online or over-the-counter sources. Every visit should include one complete list with product, strength, dose, frequency, reason, prescriber, start date, and actual administration pattern.

"As directed" is not enough

Write the exact dose and schedule. Note whether a tablet is split, whether a liquid concentration changed, whether food is required, and which medications are as-needed. Store original labels.

Supplements and CBD belong on the medication list

Include joint products, fish oil, vitamins, probiotics, herbal products, calming products, CBD or cannabis derivatives, topical creams, ear and eye products, and dental additives. Potential interactions, side effects, quality variation, and duplicate ingredients are relevant to prescribing and anesthesia.

Use a closed-loop medication system

1

Prepare

Use a weekly organizer only when the product is stable outside its original package and the veterinarian or pharmacist considers it appropriate.

2

Administer

Record doses in one shared log. Do not double a missed dose unless the prescribing team instructs you.

3

Observe

Track appetite, vomiting, diarrhea, stool color, thirst, urination, sedation, agitation, weakness, bruising, and the symptom the medicine is intended to improve.

4

Reconcile

At every visit, compare the clinic record with what is actually given at home. Remove discontinued products and confirm refills before running out.

Do not use leftover or borrowed medication

A drug prescribed for another dog, a human, or a previous condition may be unsafe because of dose, concentration, organ function, interactions, or the current diagnosis.

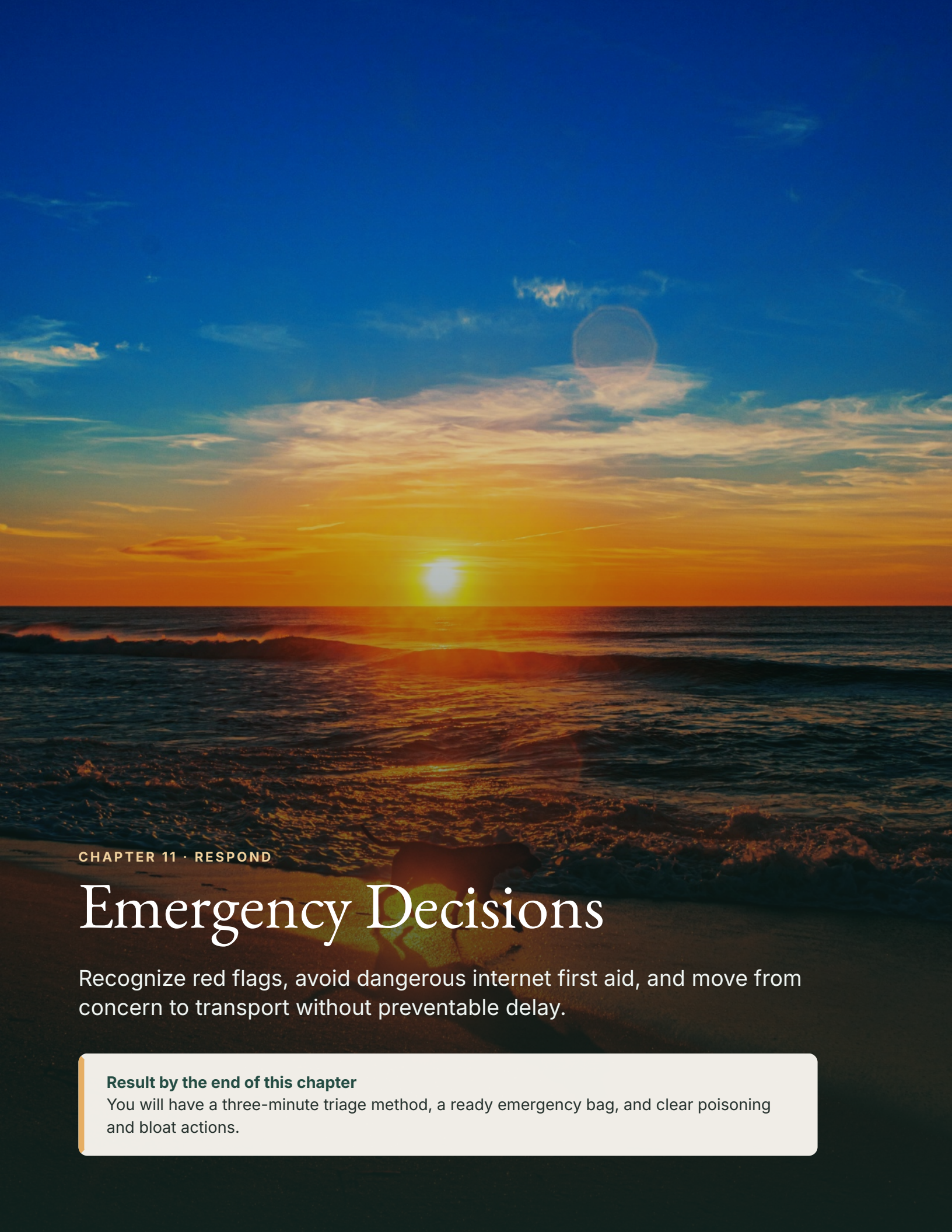
Protect identification and continuity

Confirm that the microchip is registered to current contact information. Keep a recent photo, medication list, key diagnoses, vaccine records, and emergency contacts available offline. This matters during travel, evacuation, hospitalization, or caregiver handoff.

CHAPTER RESULT

Prevention and medication are managed as one coordinated system.

You know what is due, what is given, why it is given, what should be monitored, and who to contact when a dose is missed or a reaction is suspected.

A full-page background image showing a sunset over the ocean. The sun is low on the horizon, creating a bright orange and yellow glow that reflects on the water. The sky is a deep blue with some wispy clouds. In the foreground, a dark silhouette of a dog is visible on a sandy beach, looking out towards the water.

CHAPTER 11 · RESPOND

Emergency Decisions

Recognize red flags, avoid dangerous internet first aid, and move from concern to transport without preventable delay.

Result by the end of this chapter

You will have a three-minute triage method, a ready emergency bag, and clear poisoning and bloat actions.

In an emergency, the first result is movement toward care.

Do not spend the decision window trying to prove the diagnosis at home.

Merck lists severe trauma, shock, poisoning, burns, breathing difficulty, ongoing seizures, loss of consciousness, severe bleeding, and severe pain among emergencies requiring immediate veterinary care.²⁸

OWNER VISUAL

The Three-Minute Emergency Decision

When a red flag is present, call the emergency hospital while you are preparing to leave.

Is there breathing difficulty, collapse, loss of consciousness, ongoing seizure, severe bleeding, major trauma, suspected poisoning, severe pain, or a rapidly swollen abdomen with dry retching?

GO NOW

Emergency hospital

Call ahead. Keep the airway clear. Transport safely. Do not delay for online advice.

CALL TODAY

Primary vet or urgent care

New persistent symptoms, marked appetite change, repeated vomiting, new pain, confusion, or rapid decline.

MONITOR

Document and recheck

Mild, stable change with normal breathing and function. Record the trend and define a time limit.

SUSPECTED POISONING

Call your veterinarian or ASPCA Animal Poison Control: (888) 426-4435. Do not induce vomiting unless instructed.

The red category is deliberately broad. Call the emergency hospital while preparing transport.

Breathing difficulty

Any obvious effort, gasping, blue or gray gums, inability to settle, collapse, or severe distress requires immediate care. Keep the dog calm, avoid pressure on the neck and chest, do not muzzle a dog in respiratory distress, and minimize handling. Call ahead so the hospital can prepare oxygen and triage.

Gastric dilatation-volvulus, or bloat

Repeated unsuccessful attempts to vomit, restlessness, drooling, abdominal pain or swelling, rapid breathing, weakness, and collapse can indicate GDV. It is an acute life-threatening emergency that requires

immediate stabilization and usually surgery. Although large and deep-chested dogs are at higher risk, do not dismiss compatible signs in another breed.²⁹

Dry retching plus a swollen or painful abdomen means leave now

Do not wait for vomiting, give food or water, massage the abdomen, or drive to a clinic that cannot manage emergency surgery without calling first.

Suspected poisoning

Call the veterinarian or ASPCA Animal Poison Control at **(888) 426-4435**. A consultation fee may apply. Have the package, ingredient, estimated amount, time, dog's weight, symptoms, and current medications ready. Do not induce vomiting unless a veterinary professional specifically directs it because some substances and clinical states make vomiting dangerous.³⁰

Seizure

Move objects away, reduce light and noise, time the event, and video from a safe distance if another person can do so. Do not place hands or objects in the mouth. Ongoing seizure activity, repeated seizures, failure to recover, breathing difficulty, suspected toxin exposure, or a first seizure in a senior dog requires urgent assessment.

Bleeding and trauma

For visible bleeding, apply firm direct pressure with clean material while arranging immediate care. Do not repeatedly lift the material to check. Major falls, vehicle impact, bite wounds, or blunt trauma can cause internal injury even when the dog initially stands or walks.

Heat illness

Heavy panting that progresses to weakness, confusion, vomiting, collapse, abnormal gum color, or seizures is an emergency. Move out of the heat and contact emergency care immediately. Follow professional cooling instructions during transport; overly aggressive or inappropriate cooling can create additional problems.

Prepare the emergency bag before the emergency

Information

Medication list, diagnoses, allergies, primary clinic, specialist contacts, emergency hospital, insurance details, and recent laboratory results if relevant.

Transport

Leash, harness, towel or blanket, rigid board if advised, carrier for small dogs, flashlight, disposable gloves, clean gauze, and waste bags.

Call before driving when possible

Confirm the facility is open, can handle the suspected emergency, and knows you are coming. For poisoning, the first phone call may change what the hospital needs you to bring or do.

CHAPTER RESULT

Your plan removes three delays: deciding, locating, and explaining.

The emergency card is complete, the route is saved, the bag is ready, and every caregiver knows that severe breathing difficulty, collapse, poisoning, ongoing seizure, major trauma, uncontrolled bleeding, and GDV signs are go-now events.



CHAPTER 12 • CHERISH

Quality of Life and End-of-Life Planning

Measure comfort across the whole life, plan before a crisis, and protect both the dog and the caregiver from avoidable suffering.

Result by the end of this chapter

You will have a weekly quality-of-life review, a list of the dog's defining joys, and an agreed crisis plan with the veterinary team.

Quality of life is bigger than appetite.

A dog may finish a meal while living with uncontrolled pain, breathlessness, panic, confusion, profound weakness, or repeated crises.

AAHA encourages structured quality-of-life review and good-day tracking while emphasizing that tools support, rather than replace, communication with the veterinary team. End-of-life care should be individualized around comfort, function, disease trajectory, family capacity, and the human-animal bond.^{31,32}

OWNER VISUAL

Quality of Life Is a Pattern Across Seven Domains

Use a weekly review to support a veterinarian conversation, not to create a false “perfect score.”



WEEKLY REVIEW

- 1 What still brings unmistakable joy?
- 2 Can the dog reach food, water, family, and toilet areas?
- 3 Is pain, breathlessness, nausea, or anxiety controlled?
- 4 Are difficult days becoming more frequent?
- 5 What would count as an unacceptable crisis?

Plan before a crisis limits your choices.

Review the seven domains together. One preserved domain should not hide severe loss in another.

Create the dog's “Top Five Joys” while life is good

List activities that produce unmistakable positive engagement: greeting a person, sniffing outside, lying in the sun, eating a favorite food, riding in the car, visiting a familiar place, or sleeping beside the family. Later, the list provides a concrete way to discuss whether meaningful life remains accessible.

Use good, mixed, and difficult days

Mark each day G, M, or D using the same definition. A difficult day may include uncontrolled pain, breathing distress, repeated vomiting, inability to stand or toilet, panic, or absence of meaningful engagement. One bad afternoon does not define a life, but an increasing proportion of difficult days changes the conversation.

Look at trajectory, not only today's snapshot

Some diseases fluctuate. Ask whether recovery after each crisis is complete, whether the interval between crises is shrinking, whether medication burden is increasing, and whether the dog can still perform basic functions without distress.

Palliative care and hospice are active care

Palliative care focuses on comfort and quality of life alongside or instead of disease-directed treatment. Hospice is a structured plan for a terminal or advanced condition, including symptom control, caregiver support, expected progression, after-hours contacts, and end-of-life decisions. It is not simply “doing nothing.”^{1,32}

Plan for unacceptable crises before they occur

1

Ask what progression is expected

Which symptoms are likely, which are treatable, and which indicate that the disease has changed stage?

2

Define the emergency boundary

Examples may include unrelieved breathing distress, repeated collapse, uncontrolled pain, inability to stand or toilet, persistent nausea, or severe panic. The individual threshold should be agreed with the veterinarian.

3

Discuss euthanasia before it is immediately necessary

Ask what the process looks like, whether sedation is used, whether home service is available, and what aftercare options exist. Information reduces fear and prevents rushed decisions.

4

Include caregiver capacity

Lifting ability, sleep deprivation, finances, work schedule, emotional health, and access to help affect whether a plan is sustainable. This is relevant information, not a moral failure.

When “more treatment” may not mean more good time

Ask what the proposed intervention is expected to improve, how quickly, for how long, with what burden, and what happens if it does not work. A treatment can be medically possible but misaligned with the dog's tolerance or the family's goals. Conversely, age alone should not deny a senior dog a procedure likely to restore comfort or function.

COMPOSITE OWNER SCENARIO

The family chose before the next crisis chose for them.

A dog with advanced heart disease still ate and enjoyed quiet time, but episodes of respiratory distress were becoming more frequent. The family and veterinarian agreed that another prolonged crisis with poor response to rescue treatment would be the boundary. Having the plan allowed the final decision to be calm, coordinated, and centered on preventing suffering.

CHAPTER RESULT

You have a shared definition of comfort, joy, and an unacceptable crisis.

Use the Weekly Quality-of-Life Review and Good-Day Calendar. Revisit the plan whenever disease, function, caregiver capacity, or the dog's joys change.

PRINTABLE OWNER TOOLKIT

Turn observation into evidence.

Print the pages you will actually use. Consistent, incomplete data is more valuable than a perfect tracker abandoned after three days.

1 30-Day Senior Care Setup

Complete one week at a time. The objective is a working system, not a home medical project.

Week 1: Define normal

- ☐ Record identity, diagnoses, medications, food, and emergency contacts.
- ☐ Weigh and photograph from above and the side.
- ☐ Film the six-view baseline mobility sequence.
- ☐ Describe normal appetite, water, bathroom, sleep, and activity.

Week 2: Protect function

- ☐ Create a traction path between essential resources.
- ☐ Assess bed entry, turning, support, and rising.
- ☐ Gate unsafe stairs and add night lighting.
- ☐ Check nails, paw hair, and vehicle access.

Week 3: Reduce preventable risk

- ☐ Begin the four-week dental training process.
- ☐ Create one medication and supplement list.
- ☐ Confirm vaccine, heartworm, flea, tick, and parasite plan.
- ☐ Check microchip registration.

Week 4: Align with the veterinarian

- ☐ Prepare the one-page appointment brief.
- ☐ Discuss screening and test frequency.
- ☐ Ask for body, muscle, pain, dental, and cognition assessment.
- ☐ Write the recheck date and urgent thresholds.

My most important improvement this month

Senior Dog Baseline Profile

DOG'S NAME

DATE COMPLETED

BREED / MIX

DATE OF BIRTH / ESTIMATED AGE

CURRENT WEIGHT

MICROCHIP NUMBER

PRIMARY VETERINARIAN

EMERGENCY HOSPITAL / PHONE

Current diagnoses and prior surgeries

Normal daily pattern

Domain	What is normal for this dog?
Food and appetite	
Water	
Urination	
Stool	
Walks and activity	
Sleep and nighttime	
Social behavior and favorite activities	

Known individual risks or veterinarian priorities

3 Medication and Supplement List

Include prescriptions, over-the-counter products, supplements, CBD/cannabis, topical products, eye and ear products, and dental additives.

Product + strength	Exact dose	Schedule	Why given	Prescriber / start date

Known allergies / prior reactions

Monitoring or laboratory requirements

Missed dose instruction

Write the prescribing clinic's instruction for each critical medication. Do not assume a double dose is appropriate.

Monthly Senior Dashboard

MONTH / YEAR

WEIGHT

Domain	Stable	Changed	Evidence / note
Appetite and food intake			
Water and urination			
Stool / vomiting			
Weight / body condition			
Muscle condition			
Mobility / pain			
Sleep / nighttime behavior			
Breathing / cough / stamina			
Behavior / cognition			
Skin / lumps / oral health			
Favorite activities / joy			

One change to discuss with the veterinarian

Weekly Mobility Function Score

0 = normal, 1 = slight change, 2 = clear difficulty, 3 = unable or distressed. Do not adjust prescription medication from the score alone.

Date	Rise	Walk	Turn	Stairs / car	Settle	Total	Next-day response

What the dog stopped doing

Videos saved

Contact threshold set with veterinarian

6 Sleeping Respiratory Rate Log

Use only when the dog is truly asleep and your veterinarian has recommended monitoring. Record effort and symptoms, not only the number.

Date / time	Breaths per minute	Cough?	Effort / restlessness?	Medication timing / notes

Emergency

Do not count while the dog is in obvious respiratory distress. Leave for emergency care.

7

Body and Lump Map

The Monthly Nose-to-Tail Check

Photograph and map every new mass. Appearance and feel alone cannot determine what it is.

RECORD

Location Use a body map

Size Measure in millimeters

Surface Smooth, ulcerated, hairless

Mobility Moves under skin or fixed

Change Compare date-stamped photos

Next step Ask whether sampling is indicated

NEW OR CHANGING MASS: VET REVIEW

#	Date	Location	Size	Change / sample result

Photo file names and next review dates

THE SENIOR DOG CARE FIELD GUIDE

61

DISHAA Behavior Change Log

Date	D: disorientation	I: interaction	S: sleep	H: house-soiling / learning	A: activity	A: anxiety
------	-------------------	----------------	----------	-----------------------------	-------------	------------

Time pattern, triggers, redirection, and video evidence

Veterinary screening plan

Questionnaire used: _____ Next review date: _____

Weekly Quality-of-Life Review

Domain	Comfortable / supported	Concern	What changed?
Comfort and symptom control			
Mobility and toilet access			
Nutrition			
Hydration			
Hygiene			
Connection			
Joy			

Top five joys

Good / mixed / difficult days this week

Our unacceptable crisis boundary

One-Page Veterinary Visit Brief

APPOINTMENT DATE

PRIMARY REASON

What changed, when, and how often?

Functional impact

Associated changes

Evidence brought

- ☐ Weight trend
- ☐ Mobility video
- ☐ Respiratory log
- ☐ Food / water measurement
- ☐ Lump photographs
- ☐ Medication list

Three priority questions

Plan, recheck date, and earlier-contact threshold



Emergency Card

PRIMARY VETERINARIAN / PHONE

24-HOUR EMERGENCY HOSPITAL / PHONE

POISON CONTROL

ASPCA APCC: (888) 426-4435

DOG'S CURRENT WEIGHT

Go now

Difficulty breathing, blue or gray gums, collapse, loss of consciousness, ongoing seizure, severe bleeding, major trauma, severe pain, suspected poisoning, heatstroke.

Rapidly swollen painful abdomen, repeated unsuccessful retching, drooling, restlessness, weakness or collapse: possible GDV.

Critical diagnoses, allergies, and medications

Transport plan and alternate caregiver

Poisoning rule

Bring the package. Do not induce vomiting unless a veterinarian or poison-control professional instructs you.

References

Accessed August 21, 2026. Recommendations should be individualized by the dog's veterinary team.

1. American Animal Hospital Association. 2023 AAHA Senior Care Guidelines for Dogs and Cats. <https://www.aaha.org/resources/2023-aaha-senior-care-guidelines-for-dogs-and-cats/>
2. Merck Veterinary Manual. Routine Health Care of Dogs. <https://www.merckvetmanual.com/dog-owners/routine-care-of-dogs/routine-health-care-of-dogs>
3. American Animal Hospital Association. Diagnostic Tests and Recommended Frequencies for Senior Dogs and Cats. <https://www.aaha.org/resources/2023-aaha-senior-care-guidelines-for-dogs-and-cats/diagnostic-tests-and-recommended-frequencies-for-senior-dogs-and-cats/>
4. American Animal Hospital Association. Nutrition, 2023 AAHA Senior Care Guidelines for Dogs and Cats.
5. World Small Animal Veterinary Association. Global Nutrition Guidelines. <https://wsava.org/global-guidelines/global-nutrition-guidelines/>
6. Merck Veterinary Manual. Overview of Nutrition: Small Animals. <https://www.merckvetmanual.com/management-and-nutrition/nutrition-small-animals/overview-of-nutrition-small-animals>
7. American Animal Hospital Association. 2022 AAHA Pain Management Guidelines for Dogs and Cats. <https://www.aaha.org/resources/2022-aaha-pain-management-guidelines-for-dogs-and-cats/>
8. Merck Veterinary Manual. Osteoarthritis in Dogs and Cats. <https://www.merckvetmanual.com/musculoskeletal-system/osteoarthritis-in-dogs-and-cats/osteoarthritis-in-dogs-and-cats>
9. Merck Veterinary Manual. Chronic Pain in Animals. <https://www.merckvetmanual.com/therapeutics/pain-assessment-and-management/chronic-pain-in-animals>
10. American Animal Hospital Association. Environmental and caregiver recommendations within the 2023 Senior Care Guidelines.
11. Merck Veterinary Manual. Heart Failure in Dogs. <https://www.merckvetmanual.com/dog-owners/heart-and-blood-vessel-disorders-of-dogs/heart-failure-in-dogs>
12. Merck Veterinary Manual. Periodontal Disease in Small Animals; Dental Disorders of Dogs. <https://www.merckvetmanual.com/digestive-system/dentistry-in-small-animals/periodontal-disease-in-small-animals>
13. American Animal Hospital Association. Anesthesia-Free Dentistry Is Not Recommended. <https://www.aaha.org/aaha-guidelines/dental-care/anesthesia-free-dentistry/>
14. Veterinary Oral Health Council. Accepted Products. <https://vohc.org/accepted-products/>
15. Merck Veterinary Manual. Noninfectious Diseases of the Urinary System in Dogs. <https://www.merckvetmanual.com/dog-owners/kidney-and-urinary-tract-disorders-of-dogs/noninfectious-diseases-of-the-urinary-system-in-dogs>
16. Merck Veterinary Manual. Diabetes Mellitus in Dogs and Cats. <https://www.merckvetmanual.com/endocrine-system/the-pancreas/diabetes-mellitus-in-dogs-and-cats>
17. Merck Veterinary Manual. Hypothyroidism in Animals. <https://www.merckvetmanual.com/endocrine-system/the-thyroid-gland/hypothyroidism-in-animals>
18. Merck Veterinary Manual. Hyperadrenocorticism in Animals. <https://www.merckvetmanual.com/endocrine-system/the-adrenal-glands/hyperadrenocorticism-in-animals>
19. American Animal Hospital Association. Special Disease Consideration: GOLPP Complex; Merck Veterinary Manual, Laryngeal Paralysis.
20. Merck Veterinary Manual. Tumors of the Skin in Dogs. <https://www.merckvetmanual.com/dog-owners/skin-disorders-of-dogs/tumors-of-the-skin-in-dogs>
21. Merck Veterinary Manual. Reducing the Risk of Cancer in Animals. <https://www.merckvetmanual.com/special-pet-topics/cancer-and-tumors/reducing-the-risk-of-cancer-in-animals>

22. Olby N, Gruen M, Araujo JA, et al. The CCDS Working Group Guidelines for Diagnosis and Monitoring of Canine Cognitive Dysfunction Syndrome. Journal of the American Veterinary Medical Association. Published Dec 24, 2025. doi:10.2460/javma.25.10.0668.
23. Merck Veterinary Manual. Behavior Problems of Dogs, cognitive dysfunction section.
24. American Animal Hospital Association. 2022 AAHA Canine Vaccination Guidelines, updated to classify leptospirosis as core. <https://www.aaha.org/resources/2022-aaha-canine-vaccination-guidelines/>
25. World Small Animal Veterinary Association. 2024 Vaccination Guidelines. <https://wsava.org/global-guidelines/vaccination-guidelines/>
26. Companion Animal Parasite Council. General Guidelines. <https://capcvet.org/guidelines/general-guidelines/>
27. Companion Animal Parasite Council. Heartworm Guidelines. <https://capcvet.org/guidelines/heartworm/>
28. Merck Veterinary Manual. Evaluation and Initial Treatment of Dog and Cat Emergencies. <https://www.merckvetmanual.com/special-pet-topics/emergencies/evaluation-and-initial-treatment-of-dog-and-cat-emergencies>
29. Merck Veterinary Manual. Disorders of the Stomach and Intestines in Dogs, gastric dilatation-volvulus section. <https://www.merckvetmanual.com/dog-owners/digestive-disorders-of-dogs/disorders-of-the-stomach-and-intestines-in-dogs>
30. ASPCA Animal Poison Control Center. Emergency Care and Poisoning Guidance. <https://www.asPCA.org/pet-care/animal-poison-control>
31. American Animal Hospital Association. How to Assess Your Senior Pet's Quality of Life. <https://www.aaha.org/resources/how-to-assess-your-senior-pets-quality-of-life/>
32. American Veterinary Medical Association. End-of-Life Care for Your Pet. <https://www.avma.org/resources-tools/pet-owners/petcare/end-life-care-your-pet>
33. American Animal Hospital Association. Anesthetic and Surgical Considerations for Senior Pets, 2023 Senior Care Guidelines.
34. Pet Nutrition Alliance. Nutrition resources and calorie tools for veterinary teams. <https://petnutritionalliance.org/>
35. American Animal Hospital Association. 2019 AAHA Dental Care Guidelines for Dogs and Cats.

Photography and illustration

All instructional diagrams were created specifically for this guide. They are simplified owner-education graphics and are not anatomical diagnostic images.

Image	Creator / source	License
Old whippet portrait	Karen Arnold, PublicDomainPictures.net	CC0 Public Domain
Dog walking at sunrise	Nicole Avagliano, PublicDomainPictures.net	CC0 Public Domain
Dog paw and hand	Karen Arnold, PublicDomainPictures.net	CC0 Public Domain
Sleeping dog	Vera Kratochvil, PublicDomainPictures.net	CC0 Public Domain
Black Labrador portrait	Karen Arnold, PublicDomainPictures.net	CC0 Public Domain
Whippet body photograph	Karen Arnold, PublicDomainPictures.net	CC0 Public Domain
Woman walking dog	Linnaea Mallette, PublicDomainPictures.net	CC0 Public Domain
Veterinary examination	U.S. Air Force photo by Airman 1st Class Kevin Tanenbaum, via Wikimedia Commons	Public domain in the United States

Editorial note: This guide is designed for owner education. It deliberately avoids treatment doses, individualized drug recommendations, product endorsements, and diagnostic claims based on home observations alone.



THE FINAL PRINCIPLE

Notice. Record. Compare. Ask. Act.

Your job is not to diagnose aging. It is to notice aging well enough that your dog still has choices.

Keep the system light enough to sustain.

A useful baseline, a weekly check, a monthly trend, and a strong veterinary partnership can protect more good days than constant worry.